جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : زينب مكرم احمد سليط
المحافظة : المنوفيه **الحي / المدينة :** الباجور **العنوان :** الباجور
رقم البطاقة : 29007291701608 **رقم التليفون :** 01121542458 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 29-07-1990 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: زينب مكرم احمد سليط
National ID: 29007291701608

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bilateral mammogram and breast ultrasound**

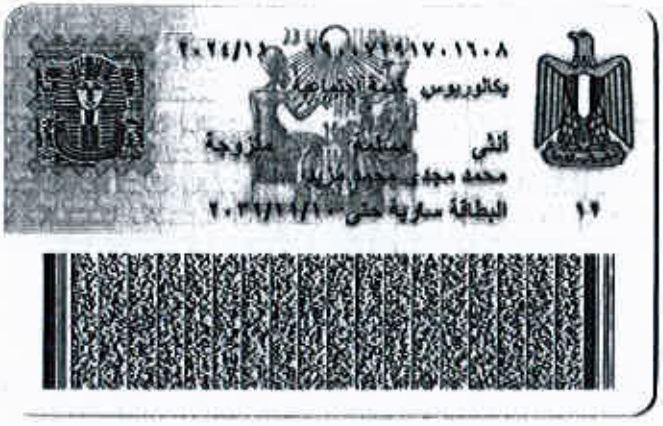
التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



Dms

ادامين يكس

نديمن



National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

01/25/2026

MDT FORM

Name:	Zainab Makram Ahmed .	MRN:	1162
Age:	35 Y	ID:	
Menopausal Sta	PRE	Phone:	01014877677
NRS SCORE:		Marital Status:	MARRIED
BMI:		Family History:	
CO Morbidity:	FREE	Reason Of MDT:	POST OPERATIVE

Clinical Data:

BX: IDC 2

ER: 8/8 **PR:** 5/8 **HER2:** 0 **KI67:** 25%

Sonomammography:

Mets Workup:

Recommendations:
PATIRNT UNDERWENT LT CENTRAL QUADREANTECTOMY AND ALND WITH FREE SURGICAL MARGINS AND 0/12 LYMPH NODES
COMPLETE PATHOLOGIC RESPONSE
STAGE: YPT0,YPN0

MDT Outcome & Plan
FOR ARAMIDIX , ZOLADEX , ZOMETA q6 MON FOR 7 YEARS

R L

TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/حنان عزت	د/ احمد نجيب				

GUSTAVE ROUSSY
INTERNATIONAL CANCER CAMPUS

Patient Name	: الأستاذه / زينب مكرم احمد سليط	Request No.	: 13001313
Patient No.	: 29007291701608	Request Date	: 17/06/2025
Path No.	: SR-825/25	Verification Date	: 21/06/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 34 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data
Retro-areolar breast mass &axillary lymph node enlargement

Nature of specimen
Block revision

Gross examination
Two block labelled by (476/25) were received
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination
1)
* Histological type: Invasive duct carcinoma
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II
* DCIS: No
* Angioinvasion: Couldn't be detected in the received biopsy
*Perineural invasion: No
2)Axillary lymph node:
Examination revealed cores of lymphoid tissue showing metastatic deposits of tumor formed of malignant epithelial cells showing moderate pleomorphism and moderate tendency to tubular formation
• IPT (Positive control showed appropriate reactivity) :
* ER: Strong nuclear reaction in 90 % tumor cells (Allred score 8/8)
* PR: Strong nuclear reaction in 10 % tumor cells (Allred score 5/8)
* HER2: No membrane reaction in tumor cells (score 0)

Verified by :

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / زينب مكرم احمد سليط	Request No.	: 13001313
Patient No.	: 29007291701608	Request Date	: 17/06/2025
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Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 34 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Ki 67: 25 % nuclear reaction of tumor cells

Diagnosis
*Retro-areolar breast mass, block &slide revision:
Invasive duct carcinoma, Grade II
IPT:
* ER: Positivee (score 8/8).
* PR: Positive (score 5/8).
* HER2: Negative (score 0).
* Ki67: Positive in 25% of tumor cells.
*Axillary lymph node:
Positive metastatic deposits

Signature
Dr. Walaa Ibrahim Ghoneim
Dr.Heba Hassan Adly

Verified by :

CAP ACCREDITED
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / زينب مكرم احمد سليط	Request No.	: 13004091
Patient No.	: 29007291701608	Request Date	: 22/12/2025
Path No.	: 3930/25	Verification Date	: 01/01/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 35 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.
Nature Of Specimen: Left central quadrantectomy & Axillary lymph nodes dissection.

Gross Examination:
* Left central quadrantectomy specimen measured 12x07x05 cm, with covering skin ellipse measured 09x04 cm. Nipple & areola looked unremarkable grossly.
* Serialling showed a tumor bed site 4.5x4x2 cm, located 1 cm from the least superior margin.
* Axillary fat measured 7x6x2 cm, entangling 12 lymph nodes, the largest measured 1.5x1x0.5 cm.
* 15 Blocks:
1- Medial margin., 2- Lateral margin., 3- Superior margin., 4- Inferior margin., 5- Deep margin., 6:10- T.B.S., 11- N&A., 12:15- ALNs.
- Gross was done by; Dr. Heba El-Shafeey.

** Frozen (Intra-operative) results:
- Free all surgical margins.

Microscopic Examination:
* Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- Cellularity (0%).
* Angioinvasion: Absent.
* DCIS: Absent
* Nipple & areola: Free.
* Surgical margins: Free.
* Axillary lymph nodes: Twelve lymph nodes free of metastatic deposits (0/12).
* Histologic response to therapy: Pathological complete response to therapy.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL

Patient Name	: الأستاذه / زينب مكرم احمد سليط	Request No.	: 13004091
Patient No.	: 29007291701608	Request Date	: 22/12/2025
Path No.	: 3930/25	Verification Date	: 01/01/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 35 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Diagnosis:
Left breast cancer (Post-NACT), Left central quadrantectomy & Axillary lymph nodes dissection.
- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.
- Surgical margins: Free.
- Axillary lymph nodes: Twelve lymph nodes free of metastatic deposits (0/12).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0.

Signature
Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

معمل الدكتور/ يوسف للتحاليل الطبية والباثولوجية

دكتور
يوسف القمص صموئيل

Dr
YOUSEF EL KOMOS SAMUEL
M. B., B.CH., M.SC., M.D.
CONSULTANT PATHOIOGIST
AT EL GALAA
TEACHING HOSPITAL

* دكتوراه الباثولوجيا- قصر العيني .
رئيس قسم تحاليل الأنسجة والأورام
والفحص الخلوي بمستشفي الجلاء التعليمي .
*استشاري الباثولوجيا الجراحية بمستشفي الهلال .
* رئيس قسم التحاليل الباثولوجية بالمعهد القومي
للكلي والمسالك البولية (سابقا) .

NAME : Zeinab Makrum Ahmed.
REFERRED BY PROF Dr :

Date : 3/ 6/ 025
LAB NO : 476/ 025

PATHOLOGY REPORT

NATURE OF SPECIMEN:

1-Tru cut retroareolar breast mass biopsy.
2-Axillary lymph node tru cut biopsy.

GROSS PATHOLOGY:

Grayish pink firm cores in each containers.

MICROSCOPIC PICTURE :

Histopathological examination revealed tumour tissue formed of infiltrating cords of malignant ductal cells surrounded with moderate desmoplasia. Lymph node biopsy showed tumour metastatic deposits.

DIAGNOSIS :

Invasive moderately differentiated ductal carcinoma with positive lymph node deposits.

Dr.Yousef E.Samuel,MD

01222275543

٢٧ شارع عمر بن الخطاب، ارض الجمعية، بجوار النقطة و بن المهندس، الدور الأرضي.
٠١٢٠٣٦١٠٢٦٩ / ٣٧١٠٧٧٢٢

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

NTERNATIONAL
EGYPT

Name: Mrs. Zainab Makram Ahmed
Date: 24/11/2025

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Left breast neoplastic lesion, post NACT assessment study. No pathology repor provided

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.
- **Left breast.**
- Scattered fibroglandular densities. (ACR b)
- Left retroareolar hypoechoic irregular shaped lesion with clips seen within measuring 12 x 7 mm. No othe Us detected suspicious lesions.
- Normal nipple/areola complex.
- Few pathologically enlarged level I axillary lymph nodes are noted , the largest measures 1.7 x 1.8 cn with increased cortical thickness 6 mm

Conclusion:-

Right breast (BIRADS 1): Normal study

Left breast (BIRADS 6): Retroareolar malignant looking lesion with clips with left axillary pathological lymphadenopathy

Best Regards,

DR. Israa Reda , M.Sc.

Mammogram
[[DOCTYPE:MAMMOGRAM]]

الشفاء للأشعة
Al Shifa Scan
للأشعة العادية (ديجيتال)
والصورات فوق الصوتية والدوبلر الملون - أليكو

Patient's name : زينب مكرم أحمد (35 Yrs)
Date: : 2025-06-02
Prof Dr. : Ahmad Nawar

REAL TIME ULTRASOUND GUIDED TRU-CUT NEEDLE BIOPSY.

⇒ Female patient 35Y, with a history of left breast abscess and a BIRAD IV lesion, and also a suspicious axillary lymph node.

⇒ On scanning, the patient was found to have a small irregular shaped, hypoechoic lesion is seen at 10 O'clock, about 4cm from the nipple and 2cm from the skin surface is noted, measures about 10x6x6mm, of ill-defined irregular borders and very fine posterior acoustic shadowing, with no remarkable internal vascularity on scanning ... ??retroareolar abscess inflammatory extension Vs BIRADS 4 lesion.

⇒ Also a large axillary lymph node was noted, measuring about 37mm longitudinal axis, with still preserved central fatty hilum, however showing fine macro-lobulation with bulky cortex ... aggressive acute on top of chronically inflamed rather than suspicious.

⇒ After explanation of the procedure to the patient and after obtaining verbal consent:

⇒ Under aseptic conditions, and using local anesthetic, US guided Tru-cut needle biopsy (18 F) was taken Two cores from the breast lesion labeled as tube I, and two cores from the axillary lymph node labelled as tube 2.

⇒ Patient was discharged from the radiology department with no complications.

With our best wishes & regards
Dr/Ibrahem Nashi, Radiology Consultant.

فرع أشمون - شما - بجوار السنترال
فرع الباجور - الشارع الجديد نزلة الكوبري
فرع سراج الضحاك - بحوار مسجد الصفي - اول البلد
0100 5340 600
0101 545 9297 - 0155 828 6399
010 323 47 888
/ AlShifaaScan

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Zainab Makram Ahmed Selett	Date : 16/06/2025
Exam : MAMMOGRAPHY BOTH	Age : 35 Year(s)
Medical ID: 29007291701608	Pat. ID: 100094

BILATERAL BREAST MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- A known case of left breast IDC, for further assessment
- Recent left breast abscess evacuation

TECHNIQUE:

- Bilateral MLO and CC views
- Bilateral breast ultrasound

REPORT:

- Scattered fibro glandular density, (ACR B), with focal parenchymal disruption seen in the retroareolar region associated with skin and trabecular thickening, owing to the previous surgical interference.
- The left UIQ shows an irregular lesion, of high density.
- Complementary U/S revealed multiple adjacent ill defined hypoechoic malignant masses, located at 10 o'clock, with the largest measuring 16x12mm.
- A residual ill defined cystic area is seen underlying the scar site, located paraareolarly.
- A large left pathological axillary lymph node is with muffled hila, measuring 4x3cm, with cortical thickness reaching 18mm.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific right axillary nodes.

OPINION:

- Left UIQ multifocal malignant process, (BIRADS 6).
- Left post operative changes.
- Normal right breast, (BIRADS 1).

Yours truly,
Dr. Mahi Mokhtar

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient ID: 86348	Patient Name English: Zainab Makram Ahmed Selett
Age: ٣٣ سنة, ١١ شهر	2025/07/27 11:14:55 AM :Report Date

Left Ultrasound guided clip insertion

Indication: Left pathological proven IDC grade (+ve ER, +ve PR and -ve Her2 and Ki67= 25%) for three clip insertions

Right breast:

- Under complete aseptic condition & ultrasound guidance three clip insertion was done for left breast as the following:
  - **Clip (1)** inserted at the index lesion located at retroareolar region
  - **Clip (2)** is inserted at most medial and deeply seated satellite noted at the UIQ 10 o'clock (Zone C) and about 3.6 cm from index lesion.
  - **Clip (3)** is inserted at most superior satellite at the UIQ 10-11 o'clock (Zone B) and about 10 mm from index lesion.
- The position of clips is confirmed by mammographic CC & ML

Best Regards,
Dr. Hedaya Nour, MD

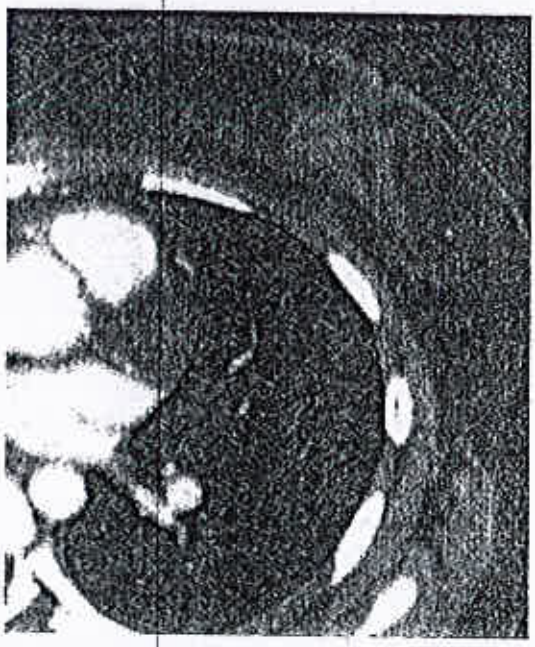
Ultrasound
[[DOCTYPE:ULTRASOUND]]

Patient Name: Zainab Makram Ahmed Selett	Date : 02/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 35 Year(s)
Medical ID: 1883/2025 2100+940	Pat. ID: 100094

POST-CONTRAST CT SCAN OF THE CHEST , ABDOMEN AND PELVIS : -

Findings:

- **Left breast mass 2 x1.8 cm .**
- **Clear both lung fields. ,**
- No evidence of pleural effusion, pleural thickening or masses seen.
- ormal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal appearance of the thoracic aorta and the great vessels.
- **Average size liver showing homogenous density. No gross focal lesions**
- **Average size spleen , no focal lesions.**
- ***Average size gall bladder , no stones .**
- *No dilated intra or extra-hepatic bile ducts .
- *Urinary bladder shows no masses or stones.
- *Normal C.T. appearance of the pancreas , adrenal glands, both kidneys ,bowel loops & other pelvic organs.



- *No retroperitoneal lymphadenopathy .
- *No ascites .
- **The examined bones show no worrisome lytic or sclerotic lesions.**

Opinion:

- ***Known case breast cancer :***
- ***No bony , pulmonary or hepatic deposits***
- ***DR/Reham khedr...MD***

Signature Dr.Mohamed Hassan

04/07/2025 4:31 PM

CT Scan
[[DOCTYPE:CT_SCAN]]